

INTEGRATING IFS WITH PHASE-ORIENTED TREATMENT OF CLIENTS WITH DISSOCIATIVE DISORDER

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INTRODUCTION

Phase-oriented treatment (International Society for the Study of Trauma and Dissociation, [ISSTD], 2011) is standard practice to protect clients with complex posttraumatic stress disorder (PTSD) and dissociative disorders (DDs), including dissociative identity disorder (DID) and dissociative disorder not otherwise specified (DDNOS; see *Diagnostic and Statistical Manual of Mental Disorders*, 4th ed., text rev.; *DSM-IV-TR*; American Psychiatric Association, 2000) from becoming overwhelmed and decompensating or activating protective parts who block and/or minimize progress. This chapter introduces a model for raising and maintaining the client's level of functioning with the tools of phase-oriented treatment in combination with the customary steps of internal family systems (IFS) treatment.

From the perspective of IFS, people with complex PTSD and DDs have exiles who carry burdens of extreme emotion and beliefs, as well as firefighters and managers who are either rigidly controlling or easily overwhelmed. This complicates the inner family of a client with DD in a number of ways. Parts tend to be more dissociated and may seem to have no connection with each other; parts are often phobic of each other and do not want anyone, including the therapist, to know about them and treatment is further complicated by the possibility of the inner family having layers of dissociated parts who reveal themselves only as progress is being made. As a result, the client who begins to function on a higher level can suddenly go into crisis. Although this may truly be a crisis, it may also indicate that progress has resulted in a "new" layer of formerly dissociated parts surfacing who may not know the client, the therapist or much about the present. Or, if they do have awareness of the present, may experience it through the eyes of an abused child.

Because the parts of clients with DD carry extreme burdens and often have difficulty accessing Self-energy, they can easily become destabilized. The first phase of treatment stabilizes and compensates for this by fostering communication and cooperation among parts and introducing coping skills. The second phase focuses on processing traumatic memories, and makes use of the coping skills to pace the process of unburdening by providing the client with more control and protection from potential destabilization. During the second phase, if the client becomes destabilized during or in between sessions, the focus of treatment returns to the first phase until the client is stable enough to continue. The third phase of treatment, in which the client adjusts to a life that is no longer controlled by trauma and neglect, is beyond the scope of this chapter and will be described only briefly.

Like phase-oriented treatment, IFS is organized in steps. The first step involves negotiating with protector parts and gaining access to Self-energy; the second involves witnessing the experiences of an exiled part; the third involves retrieving that exiled part from the past and unburdening its harmful beliefs and extreme feelings; the fourth involves inviting positive qualities to replace burdens and checking back with protectors for questions or concerns. The case of Mary illustrates the integration of a phase-oriented approach with these steps in IFS.

MARY: INITIAL CASE INFORMATION

Mary was a 36-year-old woman, twice divorced, who had three children. Her first husband was an alcoholic who physically abused her; her second husband had a gambling problem. Mary had previously worked as a teacher's assistant but had gone on disability due to "emotional problems." Although Mary could speak knowledgably about parenting, she was unable to say no to her children, who had as a result developed many behavioral problems. The children complained that Mary often contradicted herself and Mary, in turn, complained that they were always trying to get away with things and made her feel crazy.

Mary reported that her father's rage had terrified her during childhood, while her mother had been "a saint" who suffered chronic pain from severe arthritis. She grew up taking care of her mother and younger siblings, and when she was 13-years-old, her father died of alcoholism. As an adult, Mary continued providing most of her mother's care, often putting her mother's needs before the needs of herself or her family.

DIAGNOSIS

When clinicians keep in mind the indications of dissociative disorders during sessions, making a diagnosis can be quite straightforward. These indications include: clients having a history of child abuse, a history of failed prior treatments, three plus diagnoses, amnesia for periods of time during childhood and/or adulthood, somatic symptoms such as headaches, parts who seem to have a separate existence, and uneven functioning (Kluft, 1999). Research also has shown that early neglect, abandonment and/or parents with attachment disorders may be the foundation of DDs and borderline personality disorder

(Lyons-Ruth, 2003). Once a diagnosis of DD is suspected, clinicians may want to clarify the diagnosis through easy-to-administer diagnostic interviews such as the Dissociative Experience Scale (Bernstein & Putnam, 1986; or see the ISSTD website, which offers other diagnostic tests and instruments). After clarifying the diagnosis, steps can be taken to integrate phase-oriented treatment with IFS. Teaching the diagnosed client's internal family how to cope and pace unburdenings can prevent complications in treatment and facilitate healing.

MARY: SIGNS OF DISSOCIATION

Although Mary had worked hard in therapy after being diagnosed with attention deficit hyperactivity disorder (ADHD), panic disorder and depression in addition to being treated for migraines and self-cutting, her two previous therapists reported that she never made a lot of progress. This, along with years on disability following a history of work, the report of her children that she contradicted herself frequently, and the indications of child abuse (father's drinking and rage) and neglect (a mother disabled with chronic pain, the role of caretaker for her siblings as well as her mother) all indicated the possibility that Mary had a DD.

Taking note of this cluster of suggestive signs, I began to ask more specific questions about dissociative symptoms, including how much she remembered from childhood. Mary reported that she would hide in her closet and “forget everything” when her father yelled and threw things. She said, “I got so good at forgetting that I once woke up at school not knowing how I got there.” I normalized this as something children do to protect themselves when they have no other options, and said that many people continue to forget chunks of time even after childhood. Relieved, Mary went on to report “spacing out” for hours and not remembering what happened when her first husband abused her. She said she still occasionally lost time when the kids were fighting and she felt stressed. She had been afraid this meant she was crazy.

TELLING THE CLIENT—INTRODUCING THE LANGUAGE OF IFS

Clients without dissociative disorders often find the language of parts and Self “quite relieving and empowering” (Schwartz, 1995, p. 90). For clients with DD, it can sometimes be different. If “not knowing” about parts has protected a client from unmanageable experiences and feelings, or made attachment to extremely dysfunctional parents possible, or helped keep secrets necessary for survival, a therapist who suddenly knows about parts may be viewed as dangerous. As one of my clients said, “It terrifies me when you talk about parts. You're not supposed to know about them.” These clients need psychoeducation and time to feel safe enough before they can acknowledge the existence of parts.

BEGINNING TREATMENT WITH MARY

Mary exhibited all the signs of having a DD, specifically DID, a DD characterized by more than one part taking over executive functioning and exhibiting areas of amnesia (see *DSM-IV-TR*). Because of this, I approached the subject of parts cautiously, saying, “Some people have a sense of a little kid part or several parts who carry anxiety and sadness from the past. How about you?”

Mary promptly replied, “Can I get rid of her? If I could get rid of her, my life would be so much better!”

I said, “We all have parts and all parts are there to be helpful in one way or another. There’s the part of me who’s hard working, the part who likes to relax, and the part who plays with the neighbors’ kids. If you had an uncomplicated childhood or you have worked through the difficulties of your childhood, there’s a flow of communication among your parts and you switch without effort from one to the other depending on what’s needed. But when you’ve had a difficult childhood the walls among parts can become like concrete and you can get stuck in one part or another. For instance, it sounds like one part of you knows quite well about setting limits with your kids but another actually stops you from saying ‘No.’”

Mary nodded. “That’s exactly what happens. It’s like there’s a battle going on in my head.”

I replied, “As we get to know your parts, we’ll understand how they’re trying to help. Then we can help them learn to negotiate and work together. That’s how change is possible.”

THE STEPS OF IFS IN PHASE 1

The IFS concept of Self as a universal, internal presence that is undamaged and compassionate (Schwartz, 1995) is beneficial in the treatment of clients with DD in the long run. Nevertheless, much early work with complex DDs may have to be done through direct access, Self-like parts or what non-IFS clinicians call the “host” or “adult.” For clients with parts who carry the burden of worthlessness and whose protectors fear hope because it is linked to the threat of yet more disappointment, the inherent optimism in the concept of Self may feel too threatening at first. Standard practice in the non-IFS, ego state treatment of DDs has long relied on fostering curiosity and compassion within and among parts, and this is a good alternate route to reach the goal of unblending and accessing Self-energy. Along the way, it is important to notice and make use of even small bits of Self-energy. For example, one of my clients could not stop hating all of the parts who were responsible for her seemingly endless array of difficult symptoms. However, she could take a first step, which involved telling them that even though she wanted to get rid of them, she understood that to heal she had to get to know them instead.

The Phase 1 goals are for parts to learn about and feel compassion for each other rather than to process trauma, and for them to become proficient with coping skills. Because parts in the internal systems of clients with DD generally do not listen unless they are addressed directly, it is important to ask all parts to listen when teaching skills. I start by teaching two coping skills, *safe space imagery* (SSI)

and *containers*. Both are imagery exercises that help evolve the client's dissociative defenses into coping skills. In addition, I teach the client to retrieve parts from the past and orient them to the present. Until clients have adequate coping skills, I give them permission to continue dissociating traumatic material.

Initially, Mary thought that our treatment, as others before, would focus on “getting the trauma out” by recounting her childhood. I explained that instead we would work first on coping skills to regulate her symptoms and we started with SSI. This exercise teaches clients to block out negative affect and traumatic intrusions, bringing the body to a state of relaxation and calm. For many clients with DD, having all parts develop safe spaces is key to stabilization (Kluft, 1988; Twombly, 2001). I instruct clients to practice SSI every day because, in my experience, this fosters overall calm, helps with daily life management, and eventually becomes a very useful resource during trauma processing.

A SAFE SPACE FOR MARY'S LITTLE GIRL PART

To assure that this exercise would benefit as many parts as possible, I said to Mary, “I’d like everyone who wants a safe space to watch and learn how to do SSI. Then everyone can help each other to do it. What kind of place or space do you and the little girl want to start with? Pick one that is in the present where no one has ever been hurt.” The little girl decided she wanted to be inside a mountain.

I continued, “Mary, let the little girl know she can settle there in the mountain, and look around with all her senses and notice everything about it that makes it safe.” Because repetition helps to strengthen SSI, after the little girl described her safe space, I repeated her words, “No one else is there and you’re surrounded by rock walls that no one can get through. Now notice more and more about how safe it is to be there.”

The little girl noticed more about what made the safe space feel safe, but suddenly noticing there was no door, felt trapped. This was a traumatic intrusion. I said, “Tell the little girl to just keep looking around with all her senses and she’ll notice something that will help.” After a few seconds, I asked: “What is she noticing?”

Mary said, “The little girl sees there’s a secret passage out that only she can use.”

I asked, “Ask the little girl to focus on the secret passage and notice what happens.”

“She notices a sweet smell of flowers,” Mary reported. “She realizes there is a secret passage out that only she can use so she feels better.”

To provide ego strengthening, I said, “Tell the little girl she’s doing great! She’s just started learning and she’s already figured out how to make her safe space feel safer.”

“She’s smiling,” Mary reported.

We continued this process until the little girl had a full body sense of being safe and relaxed. Then, through Mary, I asked if I could talk to other parts while the little girl relaxed in her safe space for 5 minutes. When she agreed, Mary said she was amazed at how much calmer she felt with the little girl in her safe space. I said, “This was a great start and we will continue to work with all parts who want a safe space, either alone or in the company of other parts.”

After the 5 minutes in which I spoke with other parts, I had Mary check in again with the little girl. She was feeling so relaxed and protected that she decided to stay put. I asked Mary to check in with her during the week and suggested that Mary remind her that she could stay in her safe space when Mary was parenting her children. As Mary got to know the little girl through the process of developing her safe space, she understood her better and began to feel Self-energy in relation to her.

Finally, we checked for concerns from other parts.

One alarmed part said, “If the little girl is relaxing, then something bad will happen.”

I said to Mary, “Invite this part to look at you. Then let her know how old you are. Tell her about the strengths and resources you have now that you didn’t have as a little girl.”

The alarmed part replied, “I don’t believe you—but the little girl can stay in her safe space for now.”

I said to Mary, “Let her know it makes sense she doesn’t believe you, and it’s OK, she can keep watching you, and checking out the way life is now. She can also ask you and me any questions she has.”

Other parts were able to notice the little girl was relaxing, and as is often true, it felt quieter inside.

CONTAINERS

The ability of clients with DD to dissociate can be converted to good use in a coping skill called *imaginary containers* (Kluft, 1988). Containers help the client get control of intensely uncomfortable symptoms and traumatic material. All containers include a commitment to work on everything stored inside when the time is right. For clients with protectors who are already using dissociation to manage strong feelings and block out memory, it is empowering to convert this symptom into a coping skill both for managing daily life and for facilitating the Phase 2 process of unburdening. Some clients will need to create a network of customized containers for their parts; others will prefer to use just one big container.

CONTAINER IMAGERY FOR MARY

I said to Mary, “Let’s take your symptom of losing time and make it into a container in which you can purposely store painful feelings and experiences until it is time to work on them.”

As the first little girl part continued to rest in her safe space, we identified another little girl part who was burdened with extreme depression and had been around for a long time. At first Mary couldn’t get near this second little girl part because a manager, concerned about Mary becoming flooded with depression, blocked her access.

“Ask the little girl to hold on to her depression so you won’t be overwhelmed by it,” I said. “Now let the blocking part know that our goal is to help this girl manage the depression in a different and easier way. If we can, then the blocking part won’t need to use so much energy on the depression. See if it’s willing to help us by allowing you a five to ten percent connection with the depressed little girl. And if you do start feeling overwhelmed, this blocking part can always reblock the connection.”

By recognizing, respecting and suggesting an upgrade to its skill, Mary made a good connection with the blocking part. As a result, it allowed Mary 10 percent access to the depressed part, enough connection to help the little girl part begin developing a container.

I said, “Ask the little girl what kind of container she wants.”

Because the girl did not know, I suggested something like a bank vault or a box, and she decided on a toy box with a big lock.

“But now she’s scared and doesn’t want to do it!” Mary reported. “She’s afraid she’ll disappear if all the depression is put away.”

I explained, “Tell her that babies aren’t born depressed. Depression comes because of what happens after they’re born. How about if she starts by putting just five or ten percent in the box to see what it feels like?” The little girl liked that idea so I went on, “Have her focus on the depression and notice that just the right amount of it goes in the toy box and, as soon as it’s in, the box locks.”

After this, Mary reported that the little girl felt relieved and lighter. “Ask her if she wants to practice being in control by putting a bit more depression in and then taking it out?” I said.

This practice helped her develop a sense of control and mastery over her depression. Once she was secure that she would not disappear and that she had choices, she decided to put 90 percent of her depression in the toy box and she then she went to join the first part who was resting in the mountain. Realizing that this second little girl had helped her throughout life by holding her depression, Mary expressed great respect for her strength.

THE PACE OF DD TREATMENT

Of note, therapy with clients with DD goes slower than standard IFS. If Mary had been a non-DD client with enough Self-energy, her depressed part could have been witnessed directly without using a container and a safe space. However, without enough Self-energy, witnessing at this stage would have resulted in Mary’s parts either becoming overwhelmed and destabilized or they would have shut her down. Developing the container helped this part develop a sense of mastery and gave her some relief, and also helped Mary access Self-energy in relation to the part. Later, when we did analyze Mary’s “depression” during witnessing in Phase 2, we found it to be a complicated combination of horror, hopelessness, pain, sexual arousal and shame, all resulting from daily sexual abuse by her father, which occurred while her mother lay in the bed beside them “not noticing.”

Before ending any session, I check back with parts who have helped. In this session, we started by appreciating the blocking manager and asked for her comments, questions and concerns. The manager said she felt lighter and that her job seemed easier. I asked Mary, “In addition to helping you with feelings in daily life, would she be willing to help manage feelings as we work with other parts?” The manager felt pleased and respected, and became the first of a team of protectors who worked together constructively throughout Mary’s treatment.

RETRIEVAL/ORIENTING

While developing coping skills, it is important to be aware that parts tend to live in different time zones or at different points in the past. In Phase 1 we orient parts to the present through *retrieval*. No matter how oriented the client with DD appears to be to the present, questioning will reveal many parts believe they are living in the past with their families of origin and continuing to be abused and neglected. Orienting them to the present can be done by providing them with concrete information about how the present differs from the past.

ORIENTING MARY'S PARTS

We began orienting Mary's parts to the present in several ways, including developing a list of facts about her current life: I am 36, it has been 15 years since I was last abused, I have lived on Main Street in Waltham, Massachusetts for 15 years, my children's names are John and Laurie, my father died 9 years ago, Joanne is my therapist who I have been working with for 3 years. This list was communicated among the parts willing to take in new information and opened her system to the possibility of change. The second and third items, *I am 36, it has been 15 years since I was last abused*, were communicated through fast forward videos that covered the interim years; this tool is effective for helping parts to relocate in the present (Twombly, 2005). For homework, Mary oriented her parts to her current reality by showing them around her house and answering all of their questions and concerns.

FURTHER POINTS ON ORIENTING/RETRIEVING

Parts of clients with DD who are stuck in the past often fear coming into the present. As one very blended client of mine used to say, "The unknown is worse than the known." To give these parts choices, along with the control they have never had, I suggest something different. "Invite them to stay in the past and look from there through your eyes around my office. Or, if they want, they can try being in my office for five minutes and then go back to the past. Or they can go back and forth whenever they want to."

One common problem is that oriented parts can reject distressed, burdened parts and try to keep them from coming into the present. In this case, distressed parts who are stuck in the past can be invited to come closer to the present by placing their safe space in a more recent time. But in either case, until all trauma work is done, parts will invariably slip back into unprocessed trauma and will need to be retrieved more than once.

If clients live with abuse currently, it is important to help their parts differentiate the present from the past. For example, the husband of one of my clients was emotionally abusive. Her exiles, in the all-or-nothing thought process of children, believed his emotional abuse was proof that they were doomed to

be as helpless and abused in the present as they had been in the past. We pointed out that although he was emotionally abusive, she never had bruises because he did not beat her. We oriented them to the client's height and age as an adult, to the car she owned, the job she had, and the money she was saving to move out.

DISSOCIATIVE DISORDERS AND MEDICATION

For most clients with DD, medication should be considered. As [Kluft \(1988\)](#) stated, every effort should be made to minimize the pain of this very painful treatment process and medication can help. Checking with parts for concerns about taking medication helps with compliance. And I also instruct my clients to invite all parts to take the medication, which, in my experience, helps it to work (see Anderson, this volume).

USING IFS WITH COUNTERTRANSFERENCE

IFS recommends that therapists prepare for sessions by checking their own Self-energy and this is particularly important with DD clients. Many issues, for example, amnesia, dissociative defenses and preverbal abuse or neglect, will be expressed nonverbally and can be noticed first in the activation of the therapist's parts. Thus, the therapist who is aware of her own parts in session can pick up valuable information about the client's parts while managing the unavoidable dynamics of transference and countertransference (see Schwartz, this volume).

SOME DYNAMICS OF MEMORY

Ethically and clinically it is important to know how to handle requests for validation from the client. Progress can be slowed down or complicated by a therapist "validating" something that is impossible to validate unless you were present when it happened. [Courtois \(1999\)](#) wrote, "The therapist (must) understand that human memory is fluid rather than static and that conditions at the time of encoding, storage/consolidation, and retrieval may affect memory retention and accessibility" (p. 275).

[Courtois \(1999\)](#) had three recommendations: first, the therapist strives to practice from a neutral perspective regarding memory; second, the therapist understands the malleability of human memory and the differences between historical and narrative truth and third, in attending to traumatic memories the therapist maintains the goal of facilitating mastery and resolution.

MARY'S QUESTIONS FOR ME ABOUT MEMORY

When Mary asked me if I believed her story, I explained that I believed in her and that we would help her parts to feel witnessed in their experiences, and that as the parts were witnessed and their burdens lifted, Mary would figure out what was important to know about her childhood.

HOW I KNEW MARY WAS READY TO MOVE TO PHASE 2

Once a client comes to sessions reporting that she has used coping skills for symptom and affect management, the process of unburdening can begin. As Mary and her internal system practiced coping skills, understanding and problem solving, she began to present examples of progress. When a part experienced anxiety, panic or guilt, Mary and other parts were increasingly able to access enough Self-energy to be curious, ask about the part's concerns, and use various coping skills. She taught her system to orient to the present: "Our dad is dead, he can't beat us anymore, he died in 1990." She taught them cognitive strategies: "What's the evidence that you're going to be killed?" And to use their safe spaces, "Remember you can go to your safe space and let me handle the kids." When Mary got caller ID on her telephone, she used it to decide when to answer her mother's frequent calls. Initially, a part felt guilty and feared abandonment if she waited. Reminders that she could rely on the adult Mary to handle phone calls allowed her to go to her safe space. Eventually, the part had the confidence to just stay there and watch (via an imagined web cam) as Mary handled her mother's phone calls. This, in turn, reinforced the part's feeling of security. Mary reported that it became easier and easier to manage the calls and she no longer felt a surge of anxiety every time the phone rang.

Mary also made progress in parenting. Using safe spaces for the young parts who got scared when her kids yelled and having help from a manager who could block the sound of the yelling, Mary was able to set appropriate limits and notice how well the children responded. We acknowledged progress regularly and noted that some burdens, including anxiety, hopelessness and helplessness, began to grow lighter. Mary was quietly becoming proud of her accomplishments. All of this indicated that she was ready to begin Phase 2 of treatment.

TITRATING UNBURDENING IN PHASE 2

Phases of trauma treatment are not linear and destabilization can occur at any point, necessitating return to Phase 1. It might be due to external events (e.g., when Mary's mother had a minor heart attack),

internal ones (as she made progress, a formerly unknown layer of parts surfaced who had no coping skills or orientation to the present), or it might be inadvertent and therapy induced (e.g., when we worked on too much traumatic material at once).

I pace work with traumatic material and unburdening so the client can maintain her highest level of functioning. Organizing the unburdening into manageable steps is key to pacing. During trauma processing, I follow the *Rule of Thirds* (Kluft & Fine, 1993). In the first third of a session, I discuss the plan for processing trauma; in the second third, I follow the plan and in the last third, I make sure the client is stable and we plan the next session.

The goal for the client in choosing an initial target is to have a successful unburdening. Fine (1991) recommended starting with “an isolated or relatively recent event of concern to the client” (p. 672). After choosing this event, I build in further protection by storing all other traumatic material in containers to reduce the possibility of flooding. In addition, Figure 5.1 offers a model for reducing the risk of destabilization by organizing the client’s internal family. In this model, all parts who are not either directly involved or being witnessed are asked to go to their safe spaces and put up sound and feeling proofing (Twombly, 2005). Parts who handle daily life activities such as work and parenting also are asked to go to their safe spaces to be protected from the impact of witnessing. The witnessing itself is done with the assistance of a part (or parts) who have enough Self-energy and who were not involved with this particular event. Once the event is fully witnessed and the exiles are unburdened, information about the event will automatically become available to the adult/ Self-like parts (Fine, 1991).

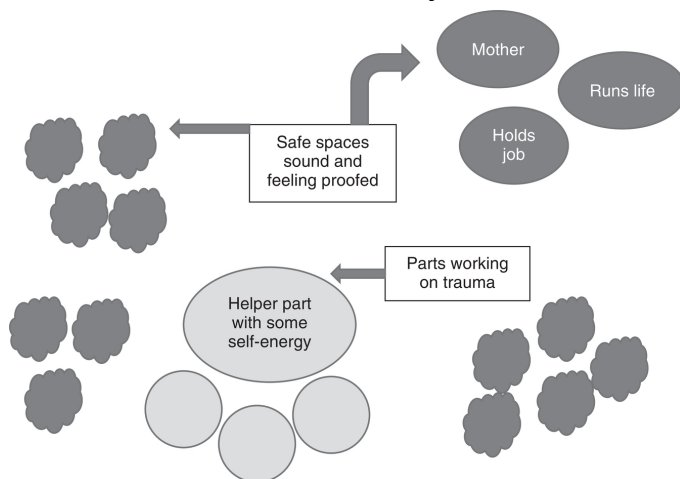


Figure 5.1 Set Up for Protected Witnessing and Unburdening Parts

Once the internal family can be organized in this way, the next step is to decide—with the client’s parts—which segment of the traumatic material to witness first. Witnessing a percentage of a large, complicated burden supports the client’s stability and sense of control. I often start with two seconds of traumatic material and increase the time as the client’s confidence grows (see Figure 5.2).

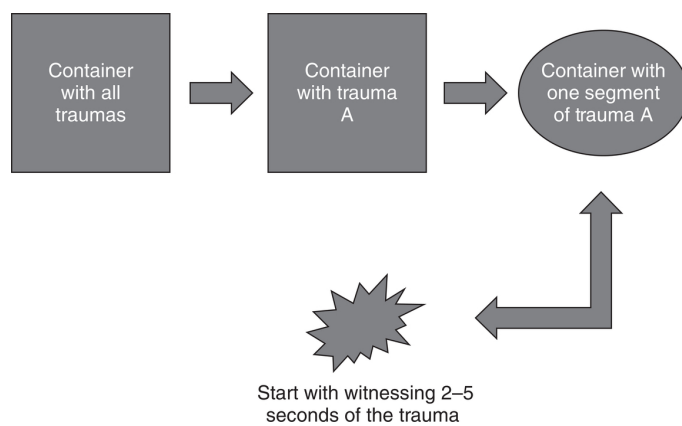


Figure 5.2 Protected Witnessing and Unburdening using Safe Spaces and Containers

MARY'S PHASE 2: WITNESSING AND UNBURDENING BY PERCENTAGES

Mary decided to begin work in Phase 2 with the memory of a teacher yelling at her for failing a test in high school. All parts who did not hold burdens from this event took the precautions described. Checking for Self-energy, we discovered Mary was blended with a critical part who said, “Mary is to blame because she’s so stupid.” Once this part felt assured that she did not have to give up believing that Mary was stupid (a burden based on old messages from her father) unless she wanted to, she was willing to relax back and watch.

This left three parts who all carried burdens from the incident. We decided to place 70 percent of their burdens in a container and to ask for one volunteer to be the spokesperson for all three. Because Mary was still concerned about becoming overwhelmed, we invited the blocking manager to stand by and block out feelings either when Mary asked her to or whenever she noticed Mary feeling overwhelmed. With this arrangement, Mary felt the confidence to begin witnessing.

The parts began to tell their story. The night before flunking the test, Mary had been awakened by sounds of her father in a drunken rage beating her mother. Mary, thinking her mother might be killed, ran out of her bedroom and tried to stop him and he turned to beating her while her mother ran out of the room. The next day Mary failed the test and her teacher threatened to drop her from the honors program. When the three affected parts felt Mary understood this portion of the incident, they were ready and able to unburden it. The realization that they could attend to and unburden such an incident without feeling overwhelmed brought them relief, pride and calm.

Her parts then decided to work on the next 30 percent, the physical pain from being beaten. When we checked for concerns, we discovered that one of the three parts believed she needed to hold on to the burden of numbing in response to pain for use in future beatings. Mary oriented this part to the present and assured her that only old feelings would be unburdened so she did not have to lose the ability to be numb. Then all the parts were able to continue with witnessing and unburdening this 30 percent. The unburdening brought such great physical relief that the numbing part decided to store her burden in a

container and take a rest. Mary, who felt relief and a new sense of inner strength, realized why the smell of alcohol was always triggering.

During the last third of this session, we shifted focus from unburdening to stabilization. The container with the remaining 30 percent of the incident was reinforced and the three parts who had been witnessed and felt tired went to rest in their safe spaces until the next session. Finally, we checked for concerns, comments and questions from the rest of Mary's system. During the following session, the remainder of this incident was witnessed and the parts were fully unburdened, which caused them to reconfigure spontaneously into one teenager. She declared, "This is so different! None of us are lost. Even though we're one, we're all here. And we feel strong!" As other parts were updated, Mary answered their concerns and questions, heard their comments and left the session feeling calmer and stronger.

PROGRESS AND THE EMERGENCE OF DISSOCIATED EXILES

Over the next several weeks, as Mary and her system spent sessions witnessing and unburdening several other incidents, her functioning became more stable than ever. Parenting was going well, she accomplished things she had been putting off, and she began socializing. Although Mary knew there was more traumatic material to work on, she wanted a break so we spent several sessions consolidating gains and talking about her progress. Then one day Mary came in reporting that she had experienced horrible flashbacks, which she had stopped with the old habit of banging her head against a wall. She felt devastated and feared that she had lost all progress.

I said, "Let's ask inside. Who can help us to understand why this is happening now?"

We discovered a part (referred to as Z) who had been deeply dissociated and had now surfaced, probably due to Mary's progress. Other parts had serious concerns about Mary having any connection with Z. But once the blocking and numbing parts agreed to act as safeguards, Mary was allowed to recognize Z's presence and offer her Self-energy. We retrieved her from the past, oriented her to the present and made her a safe space.

Next session, we prepared to unburden Z. Because many parts were extremely concerned that Z could destabilize Mary, we decided to leave Z in her safe space. The participating parts agreed to a plan in which the teenager would take on 5 percent of Z's burden and be witnessed by Mary. As always, all other parts went to their safe spaces, and all other traumatic material was left in the container.

As Mary began witnessing, the teenager described a scene in which Mary's mother watched passively while Mary's father molested her. Even with all the preparation, some protectors blended repeatedly, using avoidance and denial to protect Mary's attachment to her "good" parent. By checking and rechecking for Self-energy, and pointing out that the blocking and numbing parts were doing what they had promised to do, we were able to continue the witnessing.

In this session with Z, the teenager unburdened a total of 10 percent. Afterward the teenager went to rest in her safe space, and we noticed that Z was also resting more comfortably in hers. Mary commented, “I knew about Z but some part believed I would die if I connected with her. I wonder why she came up now?”

I suggested, “When you make progress, parts who’ve been deeply dissociated can start surfacing. Your stability seems to signal to these parts that you’re now in a place where you can help.”

As Mary’s internal family managed these initial unburdenings, her daily life functioning continued to improve and she felt confident enough to get a job during school hours with a nonprofit organization. Many parts still needed help and crises still arose, but at this phase of treatment Mary had enough Self-energy to cope with them more effectively.

POSSIBLE CHANGES AFTER PROCESSING AND UNBURDENING TRAUMATIC MATERIAL

Clinicians and clients will notice that dissociative barriers change as traumatic material is resolved and the client’s system of parts reconfigures. This might involve parts who were once very active taking a back seat as others become prominent, or it might involve groups of parts integrating into one part (like Mary’s teenager).

PHASE 3: INTEGRATION AND REHABILITATION

Once parts have become unburdened, clients transition to a life that is not controlled by dissociation and burdens from the past. Groups of parts may integrate into one and the client has access to Self-leadership. Most clients with DD have at least underperformed at work and have been in dysfunctional relationships, including ineffective, counterproductive psychotherapy relationships. This transition involves grieving all that was missed and lost. Relationship patterns usually become a focus. Many clients find that couple or group therapy is helpful while they negotiate interpersonal changes. Eventually, the client and therapist can look back on the whole treatment and terminate.

CONCLUSIONS

This chapter introduces a model for integrating IFS with the benefits of phase-oriented treatment of trauma disorders. The goal of this integration is to enable clients who would otherwise become destabilized to maintain their highest level of functioning during the treatment process, which makes IFS treatment safer and more productive for clients who have dissociative symptoms, complex PTSD and

dissociative disorders. Clients who are dissociative and whose managers cope with the feelings of parts by blocking, numbing, avoiding and dissociating, often stop making progress in standard IFS treatment. Phase-oriented treatment teaches those managers more effective ways of coping and enables progress.

Although IFS clinicians are trained to assess the client's Self-energy throughout a session, those without specific training in dissociation are usually less familiar with the diagnostic indicators of dissociative disorders and less practiced at assessing a client with DD's ability to witness extreme traumatic material. The initial IFS step, which involves negotiating with parts and gaining access to Self-energy, can be accomplished in Phase 1 with the addition of several tools: the assessment and diagnosis of a DD along with cautious use of the language of parts; getting to know parts through direct access; developing Self-energy in the form of cooperative, caring relationships among parts and with the therapist; teaching parts to convert dissociative symptoms into coping skills including safe space imagery and containers and retrieving parts from the past and orienting them to the present.

In Phase 2, the second and third IFS steps, which involve witnessing the experiences of parts, retrieving them from the past and unburdening harmful beliefs and extreme feelings, can be safely accomplished with attention to several particulars. Witnessing and unburdening only commence when the client demonstrates stability and self-assertion. When the client becomes destabilized during Phase 2, the therapy returns to the stabilization focus of Phase 1. And, finally, the intensity of witnessing and unburdening is titrated with safe space imagery and containers, while the content of containers is processed incrementally, by percentages.

Integrating IFS with phase-oriented treatment for dissociative disorders, as suggested here, may at first appear to be slower. Nevertheless, this approach can help avoid hospitalizations and excessive periods of under functioning as well as therapeutic impasses. In this way, applying a few additional tools consistently and with patience can save time and money while improving the chance of therapeutic success.

APPENDIX

International Society for the Treatment of Trauma and Dissociation: www.ISST-D.org

Recommended reading: Adult Treatment Guidelines: www.isst-d.org/jtd/2011AdultTreatmentGuidelinesSummary.pdf

Also recommended: *Coping with Trauma-Related Dissociation: Skills Training for Patients and Therapists* by Boon, Steele, and van der Hart

DIAGNOSTIC TESTS

The Dissociative Disorders Interview Schedule and the Dissociative Experience Scale are located at www.rossinst.com.

Somatoform Dissociation Questionnaire (SDQ) 20 and 5 (Nijenhuis) are available at: www.enijenhuis.nl

REFERENCES

- American Psychiatric Association. (2000). *Diagnostic and statistical manual of mental disorders* (4th ed., text rev.). Washington, DC: Author.
- Bernstein, E. M., & Putnam, F. W. (1986). Development, reliability, and validity of a dissociation scale. *Journal of Nervous and Mental Disease*, 174, 727–735.
- Courtois, C. A. (1999). *Recollections of sexual abuse: Treatment principles and guidelines*. New York, NY: Norton.
- Fine, C. G. (1991). Treatment stabilization and crisis prevention: Pacing the therapy of the multiple personality disorder patient. *Psychiatric Clinics of North America*, 14, 661–676.
- International Society for the Study of Trauma and Dissociation. (2011). Guidelines for treating dissociative identity disorder in adults, third revision: Summary version. *Journal of Trauma and Dissociation*, 12 (2), 188–212. Available at www.ISST-D.org
- Kluft, R. P. (1988). Playing for time: Temporizing techniques in the treatment of multiple personality disorder. *American Journal of Clinical Hypnosis*, 32, 90–98.
- Kluft, R. P. (January, 1999). Current issues in dissociative identity disorder. *Journal of Practical Psychiatry and Behavioral Health*, 5 (1): 3–19.
- Kluft, R.P., & Fine, C.G. (1993). *Clinical perspectives on multiple personality disorder*. Washington, DC: American Psychiatric Press.
- Lyons-Ruth, K. L. (2003). Dissociation and the parent–infant dialogue: A longitudinal perspective from attachment research. *Journal of the American Psychoanalytic Association*, 51 (3), 883–891.
- Schwartz, R. C. (1995). *Internal family systems therapy*. New York, NY: Guilford Press.
- Twombly, J. H. (2001). Safe place imagery: Handling intrusive thoughts and feelings. *EMDRIA Newsletter*, 6 (Special Edition), 35–38.
- Twombly, J. H. (2005). EMDR for clients with dissociative identity disorder, DDNOS, and ego states. In R.Shapiro (Ed.), *EMDR solutions*. New York, NY: Norton.